

Surgery to remove the lymph nodes for melanoma

Sometimes melanoma that has spread to the lymph nodes can be removed with an operation.

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Surgery to remove the lymph nodes

If [melanoma](#) has spread to your [lymph node](#), your doctor may talk to you about surgery to remove them. This operation is called a lymph node dissection or lymphadenectomy.

Having lymph nodes removed can cause side effects. It is quite a big operation and can leave a large scar. It can also cause, or put you at higher risk of, lymphoedema. [Lymphoedema](#) is chronic swelling in an area near the area of surgery, usually an arm or leg.

Your [cancer team](#) will talk to you about these risks before you make a decision about surgery. They will explain other treatments that may also be helpful or that are less likely to have risks for you.

This operation is sometimes used if melanoma has spread to the following places:

- Nearby lymph nodes ([stage 3 melanoma](#)). If this happens, and you cannot have treatment with targeted or immunotherapy drugs, or you have already had these treatments you may have surgery. The aim is to remove the cancer cells. This reduces the risk of melanoma coming back in the same area and causing symptoms.
- Lymph nodes further away from the melanoma ([stage 4 or advanced melanoma](#)). The aim is to reduce any symptoms caused by the cancer in the lymph nodes, such as discomfort or pain.

Your surgeon will remove your lymph nodes under a general anaesthetic. You are likely to be in hospital overnight and may need to stay in for 3 to 5 days.

After your operation

After the operation, you may have a small tube (drain) in place. This lets any fluid that builds up around the wound drain out of the body into a small bottle.

The nurse will remove the tube when most of the fluid has drained away. This is usually within a few days. You might go home with the drain in place. It can be removed by a practice nurse at your GP surgery or a district nurse at home.

The wound will be covered with a dressing. Your stitches or staples will be removed 10 to 14 days later. If you have dissolvable stitches, these will not need to be removed.

You will probably have some discomfort or pain afterwards, but you can have regular [painkillers](#) to help with this.

After your operation, you may see a [physiotherapist](#). They will show you some exercises to help you move normally again.

Before you go home, your nurse will give you advice about looking after the wound. They will also tell you who to contact if you have any problems or signs of infection including:

- wound redness or swelling
- the wound feeling painful or hot to touch
- pus leaking from the wound.

If you develop an [infection](#) this can be treated with antibiotics.

Some people get a small collection of fluid around the wound scar. This is called a seroma. It usually goes away within a few weeks. Sometimes your surgeon may need to drain the fluid with a needle and syringe.

It may take some time to recover following this operation. Your surgeon or nurse will explain what to expect. They will give you advice about driving, heavy lifting and other activities.

Further treatment

After surgery to remove the lymph nodes, your doctor will explain any further treatment you need. These may include [targeted therapy](#) or [immunotherapy](#) drugs to:

- reduce the risk of [melanoma coming back](#)
- treat the melanoma if it was not possible to remove all the cancer during surgery
- [treat melanoma in other areas of the body](#).

You may have [radiotherapy](#) if the surgeon is not able to remove all the cancer in the area of the lymph nodes. This is to treat any remaining cancer.

About our information

References



Below is a sample of the sources used in our melanoma information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Michielin O, van Akkooi ACJ, Ascierto PA, et al. Cutaneous melanoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. *Annals of Oncology*. 2019; 30, 12, 1884-1901 [accessed May 2022].

Michielin O, van Akkooi ACJ, Ascierto PA, et al. ESMO consensus conference recommendations on the management of locoregional melanoma: under the auspices of the ESMO Guidelines Committee. *Annals of Oncology*. 2020; 31, 11, 1449-1461 [accessed May 2022].

Peach H, Board R, Cook M, et al. Current role of sentinel lymph node biopsy in the management of cutaneous melanoma: A UK consensus statement. *Journal of Plastic, Reconstructive & Aesthetic Surgery*. 2020; 73, 1, 36-42 [accessed May 2022].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Professor Samra Turajlic, Consultant Medical Oncologist.

With thanks to: Dr Stephanie Arnold, Consultant; Kerry Jane Bate, Advanced Nurse Practitioner; Dr Philippa Closier, Clinical Oncologist; Sharon Cowell-Smith, Macmillan Advanced Nurse Practitioner Skin Cancers; and Dr Benjamin Shum, Medical Oncologist.

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Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
-

explain medical words

- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 October 2022 | Next review: 01 October 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help